



Clinic AI Starter Kit

Forward Arrow Services

Practical, controlled, human-first AI guidance for small practices and clinical teams.

Competence, it turns out, still matters.™

HIPAA-aware. Practical. Handout-ready.

What this kit is for

This starter kit is built for clinics that want to explore AI without turning patient care, compliance, or operations into a guessing game.

The goal is not to chase hype.

The goal is to take small, controlled steps that improve efficiency while protecting patient privacy, documentation integrity, and regulatory compliance.

This kit is designed to be:

- HIPAA-aware by default
- Actionable in real clinic environments
- Simple enough to hand to staff and leadership

Core principle

AI may assist the work.

AI does not replace:

- Clinical judgment
- HIPAA obligations
- Human review
- Accountability for patient outcomes

Section 1: HIPAA and AI - plain language

Before using any AI tool, understand this:

- If a tool touches Protected Health Information (PHI), it must be treated like any other vendor handling patient data
- Most public AI tools are not HIPAA-compliant by default
- Entering PHI into an unapproved tool may be considered a data disclosure event

PHI includes (not exhaustive):

- Patient names
- Dates of birth
- Medical record numbers
- Appointment details tied to identity
- Insurance or billing identifiers
- Any combination of data that can identify a patient

Bottom line:

If you would not paste it into a public email or website, do not paste it into an AI tool.

Section 2: What AI is safe to use for (starting point)

These are lower-risk, compliance-friendly uses when no PHI is included:

- Drafting internal policies and procedures
- Creating staff training materials
- Writing non-patient-facing communications
- Summarizing public medical guidelines (for internal understanding)
- Building checklists and workflows
- Drafting general patient education content (review required)

Section 3: Use with caution (controlled environments only)

These require review, controls, and often vendor validation:

- AI-assisted documentation tools
- Patient communication drafting
- Intake or triage support systems
- Scheduling automation tied to patient systems
- Revenue cycle assistance tools

Requirements before use:

- Defined human review step
- Clear data boundaries
- Vendor compliance posture reviewed
- Leadership awareness and approval

Section 4: Do NOT use without formal approval

These represent high-risk or non-compliant behavior:

- Entering PHI into public AI tools without approved safeguards
- Staff using personal AI accounts for clinic-related work
- Auto-sending AI-generated patient communication
- Accepting AI clinical output without human validation
- Using browser plugins or tools that access EHR data without review

Section 5: Five required questions before using any AI tool

1. Does this involve PHI or regulated data? If yes, stop and escalate for review.
2. Is there a Business Associate Agreement (BAA) in place? If not, the tool should not handle PHI.
3. Who is responsible for reviewing the output? A named human must be accountable.
4. Where does the data go? Storage, retention, and training use must be understood.
5. What is the failure impact? If the output is wrong, who catches it and how?

Section 6: Minimum clinic AI rules (handout-ready)

These should be posted or shared with staff.

Non-negotiable rules:

- Do not enter patient data into unapproved tools
- Do not use personal AI accounts for clinic work
- Always review AI-generated content before use
- Do not treat AI output as clinical advice
- Only use tools approved by leadership
- When unsure, stop and ask

Section 7: Quick approval checklist (1-page use)

Use this before any pilot.

Purpose and ownership:

- Clear use case defined
- Named owner assigned
- Pilot scope limited

Compliance review:

- PHI exposure evaluated
- BAA confirmed (if applicable)
- Vendor terms reviewed
- Data storage/retention understood

Operational safety:

- Human review step defined
- Staff trained on proper use
- Output quality spot-checked
- Rollback plan defined

Section 8: Practical starter pilots (low risk)

1. Policy drafting support - AI creates first drafts, leadership reviews, approved version published.
2. Staff training materials - AI helps build onboarding guides, validated by supervisors.
3. Internal meeting summaries - No PHI, AI cleans notes, team reviews.
4. General patient education content - AI drafts, clinician reviews, publish if accurate.

Section 9: Warning signs of compliance risk

- Staff using AI tools leadership did not approve
- No one knows where data is stored
- PHI is being entered casually into tools
- AI output is trusted without review
- Vendors cannot clearly explain compliance posture

Section 10: Simple clinic stance (use this verbatim)

Our clinic uses AI to support administrative efficiency and consistency. > We do not use AI to replace clinical judgment, compromise patient privacy, or bypass established compliance requirements.

Section 11: First 30-day rollout plan

Week 1:

- Assign AI owner
- Identify one low-risk use case
- Define "no PHI" boundary clearly

Week 2:

- Review tools and vendor posture
- Create temporary guardrails
- Brief staff

Week 3:

- Run controlled pilot
- Review outputs daily
- Document issues and benefits

Week 4:

- Decide: stop, adjust, or expand
- Formalize what worked
- Communicate next steps

Section 12: Closing message

A clinic does not need to move fast to benefit from AI.

It needs to move carefully, clearly, and in compliance.

That is how trust is maintained.

That is how risk is controlled.

That is how real value is created.

Forward Arrow Services

Human-First AI Guidance for Real-World Clinics

Practical implementation. Compliance-aware. Grounded in reality.